جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : زينب رشاد عبد الخالق طلبه
المحافظة : الجيزه **الحي / المدينة :** بولاق الدكرور **العنوان :** سيد راشد
رقم البطاقة : 27701122102521 **رقم التليفون :** 01149862703 **تاريخ تسجيل الإستمارة :** 11-10-2026
العمر(تاريخ الميلاد) : 12-01-1977 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج هرموني خط ثاني مثبطات الاروماتيز [14161] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: زينب رشاد عبد الخالق طلبه
National ID: 27701122102521

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Second Line Hormonal Treatment Aromatase Inhibitor

التاريخ : 05-07-2026	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



زينب
رشاد عبد الخالق طلبه
٣ ح سيد راشد محمد عثمان ناهيا
بولاق الدكرور - الجيزه

٢٧٧ ٠١ ١٢ ٢١ ٠٢٥ ٢١

KG4786454



٢٧٧٠١١٢٢١٠٢٥٢١ ٢٠٢٣/١١
ربة منزل

أنثى مسلمة متزوجة
ناجى عباس ابوالحجاج ابراهيم
١ البطاقة سارية حتى ٢٠٣٠/١١/٠٤



National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

3- Medical number: 83067117

A-Patient ID:
1- Name : زينب رشد عبدالخالق طلبه
2-ID number: 27701122102521

Sex: Female

Patient Age : 46 Y

Post-NAC
no palpable

11/7/2022

MDT

Family history		Phone	01149862703
BMI	obese II	NRS Score	1+2
GROUP		marital status	
Menopausal status		Co morbidity	
Date of MDT	05-Oct-2022	REASON OF MDT	POST CHEMO

Clinical data

Breast Imaging :

Mets Workup :

BX:	IHC:	ER:	PR:	Her 2 Neu :	KI67 :
LT IDC NST GII RT IDC NST GII 3/2022		6/8 8/8	8/8 8/8	+VE -VE	60% 10%

T	N	M
3	1	0

Sonomammography

4/2022 LEFT BREAST MALIGNANT MASS LIKELY MULTI FOCAL WITH IPSILATERAL AXILLARY LYI
BIRADS V &
RIGHT BREAST UOQ AREA OF CLUSTERED MICROCYSTS BIRAD III FOR FU AFTER 6 MNOTHS

Recommendations:

BILATERAL CA BREAST RT --> T1N0MO /// LT--> T3N1M0 RECEIVED NACT 18 WK TAXOL/CARE
LAST ONE 18/9 AND 4 CYCLES HP
SONOMAMOGRAPHY--> LT PARTIAL RESPON RT COMPLETE RESPONSE
PA U/S--> BRIGHT LIVER PT HAS INCREASE IN LES BUT IMAGING AND VIROLOGY FREE
ALT 94.9 AST 66.7

MDT OUTCOME & PLAN

FOR right WLE +SLNB & LT MRM after patient counselings to delay reconstruction

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	
SURGERY		PATHOLOGY	
Psychiatric		Nutrition	

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: زينب رشاد عبدالخالق يسريه	
MR. : 5900	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

ARAMIDEX + SUPP >> 3M ZOLADEX STOPPED SINCE 10\2025 >> E2 2,6 FSH 2,14) AFTER CONSULTING DR KHALED ABDELAZIZ TO REPEAT E2 , FSH NEXT MONTH

a case OF BREAST CANCER ON ADJ HT AI ZOLADEX STARTED 10/2022 TODAY FOR ARIMIDEX FOR 3 MONTH

A CASE OF BREAST CANCER ON HT E2 LEVEL 28/4/2026 : LESS THAN 10 FSH 2.1 TODAY FOR AI FOR 3 MONTH

تحريرا في ,

التاريخ :	19-09-2026	**الوقت :**	

الطبيب المعالج : nada mohamed elsayed

يعتمد ,

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

لست المصرية
هي
صحة
مصر

MDT FORM 30/11/2022 G

Name	زينب رشاد عبد العالم	MRN	٧٢٦
		ID	٠٠٠٠
Age	٤٥ سنة	Phone	01149862
Menopausal status		Marital status	١١٤٦١٨٠٤١
NRS SCORE	٤	Family history	٢٧٧٠١١٢٢١٠٢٥
BMI	obese	REASON OF MDT	Post op.
CO morbidity			

Clinical Data

BX: IDC Grade

ER:	PR:	HER 2:	Ki67:
Rt 8/8	8/8	-ve	60%
Lt 8/8	8/8	+ve	10

Sonomammography:



Mets Workup:

Pt received 6 TCHP

Rt T N M
Lt T3 N1 M0

Recommendations: Post op. For TIDT. Post NACT

Rt br → WLE + SLN ⟹ IDC grade II - Pathological partial respons
Free margin - Free SLNs (0/5)

Lt br → MRM ⟹ Pathologic complete response in a previously diagnosed
carcinoma - Free - nipple & areola - free margin.

MDT OUTCOME & PLAN 6-12-2022

bilat PORT / Cont / yearly f/u / Tt for 5-10 y

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatri
SP 6/12/2022		Dr 6/12/2022			

MDT Form
[[DOCTYPE:MDT_FORM]]

مبادرة رئيس الجمهورية
تدعم صحة المرأة المصرية

MDT FORM

27/4/2022

Rt. ant. & breast hyperplasia

Name	زينب ...	MRN	١١٢٩/٠٠٠٠
		ID	٢٧٧٠١١٢٥٠٠٥١
Age	٤٣ ...	Phone	
Menopausal status		marital status	
OCP		Family history	
Co morbidity	Diabetic		

Clinical Data

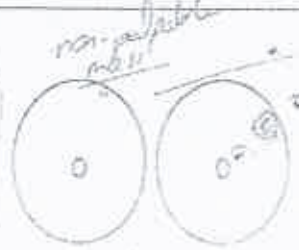
Bx: IDC GR II

ER:	PR:	HER 2:	KI67:
8/8 (Rt) 8/8 (lt)	8/8 (Rt) 8/8 (lt)	+VE −VE	60% 10%

Triple +VE

Sonomammography:

Mets Workup:



Bilateral IDC GR II
Rt $T_1N_0M_0$
lt $T_3N_1M_0$

Recommendations:

US to determine Intraductal Extension to the Nipple before deciding for the type of surgery.

Neoadjuvant Herceptin and CTh first followed by surgery according to the response

MDT OUTCOME & PLAN

Clipping of both lesions Rt & lt (US) done

Start NACT (TCHP x 6) guided by clinical response then consider Rt. CWLE & SLND and Lt. BCS ± IORT & Reconstruction according to response & extent of ductal extension

Target US revealed

SIGNATURE			
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY

Related to patient call for from the ma the nipple =

MDT Form
[[DOCTYPE:MDT_FORM]]

كلية الطب

Faculty of Medicine

القصر العينى

قسم الباثولوجى

وحدة تشخيص أمراض الثدى

IMMUNOHISTOCHEMISTRY REPORT

Name: زينب رشاد عبد الخالق **Pathology Number: 625 i- 2022**

Age: 45 Y **Patient's I.D: 27701122102521** **Date: 02/ 03/ 2022**

CLINICAL DATA: A case of *bilateral* breast cancer, for estrogen, progesterone receptors, Her-2/neu and Ki-67.

PROCEDURE: Sections from paraffin blocks were treated with antigen retrieval solution and then treated with monoclonal antibodies for estrogen, progesterone & Her-2/neu, (CONFIRM™ anti-Estrogen Receptor (ER) (SP1) Rabbit & CONFIRM™ anti-Progesterone Receptor (PR) (1E2) & CONFIRM™ anti-HER-2/neu (4B5) Primary Antibody), CONFIRM anti-Ki67 (30-9) Primary Antibody), using Ventana IView™ system. DAB was used as a chromogen and hematoxylin as a counter stain.

RESULTS:

Left	***Right***
Examination of section treated for estrogen receptors revealed strong positive nuclear staining in most of tumor cells by chromogen.	Examination of section treated for estrogen receptors revealed strong positive nuclear staining in most of tumor cells by chromogen.
Examination of section treated for progesterone receptors revealed strong positive nuclear staining in most of tumor cells by chromogen.	Examination of section treated for progesterone receptors revealed strong positive nuclear staining in most of tumor cells by chromogen.
Examination of section treated for Her2/neu revealed strong complete membrane staining in >10% of tumor cells by chromogen (score 3+).	Examination of section treated for Her2/neu revealed negative membrane staining of tumor cells by chromogen (score 0).
Examination of section treated for Ki-67 revealed positive nuclear stain in average of 60% of tumor cells.	Examination of section treated for Ki-67 revealed positive nuclear stain in average of 10% of tumor cells.

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient name: Zeinab Rashad Abd EL-Khalek

Date:12/5/2022

Indications

Patient had bilateral pathologically proven malignant mass, with right breast small enhancing lesion, being clipped

Bilateral size discrepancy, with right breast is much smaller than the left one, with no related surgeries

bilateral breast ultrasound

Report:

- Homogenous background echotexture, fatty.
- Left breast UOQ two irregular spiculated hypoechoic masses, seen along 2 O'clock, the main one, is seen 13 cm from the nipple, measured 3.7 X 2.9 cm
- The other one is 5.3cm apart, measured 3.1 X 2.6 cm, being 3.5cm from the nipple, with ?? intra-ductal extension reaching the nipple, with ?? retracted left nipple
- the parenchyma in between the masses is unhealthy hypoechoic parenchyma, likely infiltrated, measured 5.3cm.
- right breast 8o'clcok isoechoic small mass measuring about 6.5 x 5.7 mm and is seen 9.3 mm from the skin, seen with ?? echogenic clip within.
- Pathological left axillary, level I and II LNs, seen with diffuse asymmetric cortical thickening and muffled hila.
- Free left supora-0clavicular LNs
- Right non specific axillary LNs

Opinion:

- Left breast UOQ 2 masses with ?? infiltrated parenchyma in between, pathologically proven malignant
- Right breast 8o'clcok clipped lesion
- (BIRADS 6)

Yours truly

Dr Engy Abdullah

Dr Hend Salah

Sonography
[[DOCTYPE:SONOGRAPHY]]

Patient Name: ZAINAB RASHAD ABD AL KHALEK TOLBA	Patient ID: 27701122102521
Study Date: 2/14/2022	Age: 45 Years

Bilateral Full Field Mammography with Complementary Ultrasound Examinations Revealed

Findings:

- Congenitally smaller rigt breast.
- **The left breast shows:**
- Scattered fibroglandular densities (ACR b).
- Three irregular dense masses are seen in the left breast UOQ, they show spiculated outlines ,architectural distortion and intercommunicating linear densities measuring collectively about 12.8 x 10 cm .
- **Complementary Ultrasound**
- An irregular hypoechoic mass with spiculated margins located at 2'oclock and measuring about 3.2 x 2.2 x 1.6 cm with subsequent distortion of the surrounding parenchyma and internal pathological vascularity. Two neighbouring spiculated lesions of similar sonographic appearance are seen at 1-2 O'clock paraareolar in location, around 3 cm from the nipple with suspected intraductal extension to the nipple. Both masses show skin and ducta suspected extension evident by prominent intercommunicating ductal system.
- A small well-defined oval snaped anechoic cyst is seen located at 3'oclock and measuring about 1 x 0.5 cm.
- Multiple left axillary pathological lymph nodes, reaching infraclavicular region (Level 3). No pathologically enlarged supraclavicular lymph nodes.
- **The right breast shows:**
- Scattered fibroglandular densities (ACR b).
- A small well-defined isodense rounded lesion is seen at the UOQ, that corresponds on ultrasound examination to 9 O'clock small oval shaped area of clustered anechoic microcysts measuring about 1 x 0.4 cm. Another small cyst is seen measuring 13x3mm.
- Normal skin thickness and contour.
- Normal nipple areola complex.
- Non specific axillary lymph nodes.

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Patient Name	الأستاذه / زينب رشاد عبدالخلق طلبه	Request No.	: 13003949
Patient No.	27701122102521	Request Date	: 27/10/2022
Path No.		Verification Date	: 13/11/2022
Sex	Female	Financial Category	: اوراد دار السلام - نفقة الدولة
Age	45 Y	Department	: Out Patient
Referreing Doc		Registered by	: .marwa mohamed youss

Pathology Report

CLINICAL DATA:
Right breast cancer, post NACT

NATURE OF SPECIMEN:
Right WLE & SLN specimens
Taken at 27/10/2022 ; 12:30 pm

GROSS EXAMINATION:
1- Right SLN: A fibro fatty tissue piece measured 4x3 cm & entangling 5 nodules up to 0.7x0.7cm.
2- Right WLE specimen measured 10x7 cm , with covering skin elipse measured 7x2 cm
- Tumor extent
  - o Number: A firm gryish white tumor bed site
  - o Size: 2.5x1 cm
  - o Skin involvement: Free grossly
- Distance from margins: 1.5 cm from inferior margin (least margin)

MICROSCOPIC EXAMINATION:
Right breast ; WLE & SLN biopsy :
- Invasive duct carcinoma
- Histological grade:grade II (3+2+2)
- Ductal carcincma in situ Scattered foci(10%)
- Skin involvement: Free
- Lymph node involvement: Free SLNs (0/5)
- Angio invasion: No
- Perineural invasion: No
- Resection Margins: Free all resected margins
- Pathologic partial response, Cellularity (30%).

Verified by :
walaa ghoniem

Room No. 10 - First Floor Building No. 257 - 26th Corniche - Nile El Qubba - Cairo - Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name: ZAINAB RASHAD ABD AL KHALEK TOLBA	Patient ID: 277011221025
Study Date: 2/14/2022	Age: 45 Years

Opinion:

- *Left breast malignant process likley multifocal with ipsilateral axillary lymphadenopthy (BIRADs V), for tissue biopsy. Further contrast study is recommended for better assessment of disease extent.*
- *Right UOQ area of clustered microcysts (BIRADS III) for follow up, after 6 months.*

Res Dr Mahmoud Arafat

Dr. Basma Alkalaawy, MD

Women imaging

Date & Time: 14/02/2022 12:05:22 P

Mammogram
[[DOCTYPE:MAMMOGRAM]]

OLBA,	Age: 45 Years
Study Date: 2/23/2022	

Contrast Enhanced Mammogram study:

Reported case of left multifocal breast malignant masses for pathological correlation. CESM is done for assessment of extent of disease.

The left breast shows:

Two intensely enhancing masses at the UOQ corresponding to the previously reported lesions. No other enhancing areas within the left breast. A malignant left axillary node is seen enhancing as well.

The right breast shows:

Faintly enhancing triangular lesion at the 8 O'clock position of the right breast about 4 cm from the right nipple IT measures about 9x7mm in maximum dimensions as seen on second -look ultrasound. IT appears as a hypoechoic mass with a circumscribed margin and minimal internal vescularity. this lesion shows faint enhancement on contrast study warranting histopathological correlation (BIRADS 4a).

No other enhancing areas within the right breast.

Right axilla shows a single node with thick cortex and preserved hilum.

Free supra and infraclavicular regions bilaterally.

Conclusion:

- Left breast multifocal malignant masses with metastatic ipsilateral axillary nodes [illegible] for histopathological correlation (core biopsy).
- Right breast faintly enhancing lesion at 8 O'clock; warranting Us guided core needle [illegible] as well (BIRADS 4a).

Thanks for referral,

Dr. Rania Hegazy, MD

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Zainab Rashad Abdel Khalek	Date : 20/09/2022
Exam : ultrasound BREAST (BOTH SIDE)	Age : 45 Year(s)
Medical ID: 1136/2022	Pat. ID: 14273

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Known case of bilateral breast cancer on therapy

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Right UOQ surgical clip with no underlying spiculated lesions.
- Subtle left UOQ asymmetry.

Complementary ultrasound :

- Left breast 1 O'clock para areolar and 2 o'clock irregular hypoechoic solid mass , measured about 5 x 15 mm and 6 x 22 mm respectively.
- No right breast spiculated lesions.
- Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
- Nonspecific axillary LNs.
- No pathological enlarged supraclavicular lymph nodes.

Opinion:

Follow up post NACTH showing:

- Left breast UOQ pathologically proven carcinoma showing partial therapeutic response (BIRADS6)
- Right breast surgical clip with no underlying lesion denoting complete response (BIRADS6)

Yours truly

Signature Dr. Aya Deabes

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Zainab Rashad Abdel Khalek	Date : 26/06/2022
Exam : C.T CHEST	Age : 45 Year(s)
Medical ID:	

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST: -

***** A know case of breast cancer:***

Findings:

- Absent pectoralis muscles on the right side.
- Bilateral multiple scattered sub-pleural ground glass opacities are seen mostly on the periphery of the lungs.
- No sizable hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- No gross cardiac abnormality. No pericardial effusion detected.
- Upper abdominal cuts show fatty liver.

OPINION:

- ***Bilateral multiple scattered sub-pleural ground glass opacities (CORADS 4), for clinical and laboratory correlation.***

Dr. Mohammed Mahjoub
Dr. Amr Farouk, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Zalnab Rashad Abdel Khalek	Date : 14/01/2025
Exam : MAMMOGRAPHY BOTH	Age : 48 Year(s)
Medical ID: 1136/2022	Pat. ID: 14273

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

- Left MRM
- Right breast conserving therapy

Report:

Left MRM:

- Operative bed shows no mass or collection
- No regional lymphadenopathy.

Right Breast:

- Predominantly fatty breast parenchyma (ACR a)
- Operative bed parenchymal distortion and scarring with no sizable mass
- No suspicious mass lesions, micro calcifications
- No sizable collection
- No regional lymphadenopathy

Opinion:

- Post Left MRM & Right BCS status showing no evidence of recurrence (BIRADS 2)

Best regards,
Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCI, Cairo University
14/01/2025 2:29 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

IMMUNOHISTOCHEMISTRY REPORT

Name: [illegible] Pathology Number: 635 I- 2022

Age: 45 Y Patient's I.D: 27701122102521 Date: 02/ 03/ 2022

CLINICAL DATA: A case of bilateral breast cancer, for estrogen, progesterone receptors, Her-2/neu and Ki-67.

PROCEDURE: Sections from paraffin blocks were treated with antigen retrieval solution and then treated with monoclonal antibodies for estrogen, progesterone & Her-2/neu, (CONFIRM™ anti-Estrogen Receptor (ER) (SP1) Rabbit & CONFIRM™ anti-Progesterone Receptor (PR) (1E2) .& CONFIRM™ anti-HER-2/neu (4B5) Primary Antibody), CONFIRM anti-Ki67 (30-9) Primary Antibody), using Ventana IView™ system. DAB was used as a chromogen and hematoxylin as a counter stain.

RESULTS:

Left	Right
Examination of section treated for estrogen receptors revealed strong positive nuclear staining in most of tumor cells by chromogen.	Examination of section treated for estrogen receptors revealed strong positive nuclear staining in most of tumor cells by chromogen.
Examination of section treated for progesterone receptors revealed strong positive nuclear staining in most of tumor cells by chromogen.	Examination of section treated for progesterone receptors revealed strong positive nuclear staining in most of tumor cells by chromogen.
Examination of section treated for Her2/neu revealed strong complete membrane staining in >10% of tumor cells by chromogen (score 3+).	Examination of section treated for Her2/neu revealed negative membrane staining of tumor cells by chromogen (score 0).
Examination of section treated for Ki-67 revealed positive nuclear stain in average of 60% of tumor cells.	Examination of section treated for Ki-67 revealed positive nuclear stain in average of 10% of tumor cells.

Immunohistochemistry
[[DOCTYPE:IHC]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
ادارة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Date : 12/11/2024	Patient Name: Zainab Rashad Abdel Khalek
Age : 47 Year(s)	Exam : ultrasound ABDOMEN& PELVIS
Pat. ID: 14273	Medical ID: 1136/2022

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- *Enlarged in size (about 19 cm at the right MCL), smooth outline, bright echopattern. No intra hepatic biliary radicle dilatation.*
- *Portal vein is not dilated and patent .*

Gall Bladder: -

- *Average size, normal wall thickness, no stones.*
- *CBD is not dilated.*

Spleen: -

- *Average size (About 10 cm), displaying homogenous echopattern.*

Kidneys: -

Are of normal size and site , normal echogenicity , no stone or backpressure changes , no solid or cystic lesion.

-Para-aortic and pancreatic area: no gross lesion

URINARY BLADDER: Normal filling. Normal wall thickness, no stones or diverticulae or solid lesion detected.

Uterus: -: anti-verted, average sized uterus. Average Endometrial thickness

No free fluid in the Douglas pouch.

Opinion:

Bright hepatomegaly.

Best Regards,
Dr Dina Gaber ,FEBR

Not Approved

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry of Health
Specialized Medical Center
Dar El Salam Cancer Hospital
Radiology Department

Patient Name: Zainab Rashed Abdel Khalek	Date : 26/10/2023
Exam : MAMMOGRAPHY BOTH	Age : 46 Year(s)
Medical ID: 1136/2022	Pat. ID: 14273

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM and right CBS in 10/2022

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- The right breast is almost entirely fatty (ACR A)
- right LOQ post-operative distortion, with no spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed clear right LOQ operative bed, with no related solid or cystic masses
- Free left operative bed with no evidence of recurrent mass lesions
- Free bilateral infra and supra clavicular regions.

Opinion:

- Left MRM with free operative bed with no evidence of recurrent mass lesions
- Right breast LOQ post CBS, with no related suspicious findings
- (BIRADS 2)

Yours truly

Signature Dr. Engy Abdallah

26/10/2023 10:53 AM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Imaging

كايرو سكان
للأشعة والتحليل
CAIRO SCAN
Radiology & LABS

Name :Zainab Rashad Abd Alkhak Tulbah
Patient ID: 562443 **Procedure date: 04/24/2022**

To^{99m} MDP BONE SCAN:

Findings:

- Abnormal small focal active osseous lesion is seen at the mid posterior part of the left 10th rib ?traumatic ?metastatic for clinical and radiological correlation
- Otherwise; the rest of the surveyed skeleton is free exhibiting normal tracer distribution pattern.
- Degenerative changes of the spine are seen.
- Osteoarthritic features of both shoulders and knees are also noted.

Opinion:

- Small single equivocal active osseous lesion detected at the mid posterior part of the left 10th rib .for clinical and radiological correlation to differentiate traumatic from early metastatic nature

Best Regards

Dr.Maha Abd Elkareem,M.D.

For any inquiries regarding the findings in the report please contact us on **radreport@cairoscan.com.eg**

19144

Bone Scan
[[DOCTYPE:BONE_SCAN]]